

--SUMMARY--

Decision No. 667/19

29-May-2020

D.McBey (*dis*) - C.Sacco - M.Ferrari

- Consequences of injury

The worker slipped and fell in February 2013. The worker appealed a decision of the Appeals Resolution Officer denying entitlement for a rotator cuff tear.

The majority of the Panel found that the worker had entitlement for the rotator cuff tear. The worker was able to continue working and did not seek treatment for one month, but there was no evidence of significance that the worker had shoulder problems prior to the accident. Even if the worker engaged in physical activities at work after the accident, this did not negate the causal connection between the fall and the rotator cuff tear.

The appeal was allowed.

The Vice-Chair, dissenting, accepted the contemporaneous evidence of the worker's continued ability to work without notable restriction after the accident, and would have denied entitlement for the shoulder injury.

24 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w2920s]

Style of Cause:

Neutral Citation: 2020 ONWSIAT 1088



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 667/19

BEFORE:

D. McBey Vice-Chair
C. Sacco: Member Representative of Employers
M. Ferrari: Member Representative of Workers

HEARING:

April 11, 2019 at Toronto
Oral
Post-hearing activity completed on October 4, 2019

DATE OF DECISION:

May 29, 2020

NEUTRAL CITATION:

2020 ONWSIAT 1088

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) R. Sheridan, dated January 19, 2016

APPEARANCES:

For the worker: Z. Robinson, Office of the Worker Adviser (OWA)

For the employer: Not participating

Interpreter: Not applicable

Workplace Safety and Insurance
Appeals Tribunal

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Toronto ON M5G 2P2

Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

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REASONS

- [1] The following are the reasons of the Panel Majority in this appeal. The Panel Majority consists of the Member Representative of Workers and the Member Representative of Employers. The dissent of the Vice-Chair is attached below.

(i) Introduction

- [2] The worker appeals a decision of the ARO, dated January 19, 2016, which concluded that the worker was not entitled to benefits for a right shoulder and right arm injury arising out of an accident on February 18, 2013. The ARO rendered a decision following an oral hearing.

- [3] Although the ARO accepted that the worker had slipped and fallen on ice at work on February 18, 2013, the claim for benefits was denied. The ARO found that the worker had delayed in seeking medical attention until March 20, 2013, and that he had continued to perform his regular duties, including overtime, without any continuity of complaint until he was advised that he required surgery for his right rotator cuff in March of 2013. This lead the ARO to conclude that the worker did not sustain a personal injury as a result of the slip and fall accident on February 18, 2013 and that therefore, the diagnosis of a full supraspinatus tear in March 2013 was incompatible with the accident history. The worker appeals this decision to the Tribunal.

- [4] At the hearing the worker's representative advised the Panel that the worker was only seeking initial entitlement for the right shoulder and not for the right arm. Any attempt to renew the appeal of the issue pertaining to the right arm will be subject to the time limit provision of section 125(2) of the *Workplace Safety and Insurance Act, 1997* (the WSIA).

- [5] The employer did not participate in the proceedings.

(ii) Issues

- [6] The sole issue under appeal is initial entitlement to a right shoulder injury as a result of an accident on February 18, 2013.

(iii) Background

- [7] The following are the basic facts.

- [8] The now 58 year old worker started as a machine operator with the accident employer, a recycling and waste management company, in July 2012. The worker claimed that he slipped and fell on ice on February 18, 2013, injuring his right shoulder and that he immediately reported the accident to his employer. The worker continued to work with no lost time believing that he had suffered a shoulder sprain, that would improve with time.

- [9] On March 20, 2013 the worker sought medical attention for the first time when he attended at a walk-in clinic. In the Form 8, of the same date, Dr. Dworak indicates that the worker fell on his right shoulder on February 18, providing a diagnosis of "Contused a torn shoulder". In the contemporaneous clinical note, Dr. Dworak indicates that the worker could not lift his arm, Mobicox was prescribed, an x-ray was negative and the worker was referred for an ultrasound. The ultrasound report of the right shoulder dated March 22, 2013 indicated a "full thickness tear of the anterior supraspinatus tendon with some associated bursal fluid and hemorrhagic biceps tenosynovitis."

[10] The employer completed its report of injury (Form 7) on March 28, 2013, indicating that the worker had reported on February 18, 2013 that he had slipped on ice at the transfer station, injuring his right shoulder but that he did not require medical attention and that he continued to perform his regular duties and overtime. The worker now advised that he required surgery for the shoulder injury. According to the Form 7, the employer was not aware of any previous similar right shoulder disability.

[11] In a follow up clinical note dated April 1, 2013 Dr. Dworak states that the worker used a loader, he had a torn tendon and a referral was made to Dr. Randle; decreased range of motion of the right shoulder and modified duties were also noted. The corresponding Functional Abilities Form (FAF) of the same date from Dr. Dworak indicated that the worker was capable of returning to work with restrictions: "Limited use of R shoulder, [the worker] mentioned that he can use his loader safely because he is not using his R shoulder while using it."

[12] In the Worker's Report of Injury (Form 6) dated May 27, 2013, the worker noted that he was right hand dominant and that he had injured his right shoulder and arm when he slipped on ice landing on his right elbow on February 18, 2013. He noted that he had delayed in seeking medical attention because he initially believed that he had sustained a sprain. The worker reported the accident immediately to his supervisor, J.B. and he identified a truck driver, B.J., as a witness. The worker included a copy of the undated witness statement from B.J. with his Form 6, which read as follows:

I [B.J.] seen [the worker] the loader operator slip and fall on the ice in the parking lot of [the transfer station] between the trailer and building on Feb. 18th about 5:00 a.m. There is no lights in that area, where rain and snow come off the roof of the building.

[13] In a report to the family doctor, dated May 31, 2013, Dr. Randle, orthopaedic specialist, states:

...As you know, he is a 52-year-old machine operator who has a history of right shoulder pain. **He fell on ice suffering a rotator cuff tear of his right shoulder.**

....

Imaging confirms a full thickness tear of his supraspinatus.

Impression and Plan: [the worker] would benefit from an arthroscopic rotator cuff reconstruction...[emphasis added]

[14] In a subsequent Form 7 dated May 28, 2013, under a new claim, the employer advised the WSIB that the worker "alleged that on May 24/13 he had an incident in the workplace that caused an increase in pain to his right shoulder...". No Form 6 was filed by the worker for this incident.

[15] In a May 27, 2013 clinical note from Dr. Harmathy, family doctor, there is no mention of any incident on May 24, rather Dr. Harmathy states: "He has slipped on some ice and landed on his right shoulder. He was seen at another hospital and x-rays were normal however he has a total thickness tear of the supraspinatus muscle involving the shoulder...." Dr. Harmathy's clinical note refers strictly to the February 18 claim with no mention of an incident on May 24, 2013. There was no Form 8 filed in this claim nor any other contemporaneous medical evidence with respect to an incident on May 24, 2013.

[16] An Eligibility Adjudicator initially allowed the May 24, 2013 claim for a right shoulder strain but reconsidered upon the employer's submission that the worker had a non-compensable pre-existing right shoulder tear. After speaking with the worker on June 18, 2013, the Eligibility Adjudicator rescinded initial entitlement finding that the increased right shoulder pain as a result of picking up a piece of metal and placing it in a bin, without any jarring or jerking of the shoulder, did not constitute a new accident. While the worker's representative filed an Intent to Object Form, as of yet this objection has not been referred to an ARO. The worker now appeals the ARO decision dated January 19, 2016 to the Tribunal.

(iv) Law and policy

[17] Since the worker claimed to be injured in 2013, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[18] An "accident" is defined in section 2(1) to include:

- (a) a wilful and intentional act, not being the act of the worker,
- (b) a chance event occasioned by a physical or natural cause, and
- (c) disablement arising out of and in the course of employment.

[19] General entitlement to benefits is governed by section 13:

13(1) A worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan.

(2) If the accident arises out of the worker's employment, it is presumed to have occurred in the course of the employment unless the contrary is shown. If it occurs in the course of the worker's employment, it is presumed to have arisen out of the employment unless the contrary is shown.

[20] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

[21] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision #9, would apply to the subject matter of this appeal: 241; 300.

(v) Testimony

[22] The worker testified that he was a heavy equipment operator who generally worked seasonally through his union. At times, during the off season, between approximately December and April, he also worked doing home renovations. When there was a shortage of union work, he also operated heavy equipment at non-union jobs.

[23] The worker stated that he was hired to work at a transfer station for the accident employer in July 2012 as a machine operator by A.B., the "big boss" in charge of all the transfer stations, whom he had worked with previously doing similar work at a different location. The worker's understanding was that he was being hired in order to clean up the transfer station because he had previous experience. It was also his understanding that he was to try and find fault with another employee who had been injured and whom the employer wished to get rid of.

[24] The worker's duties involved running the loader to fill the compactor with mixed garbage that would be taken away by approximately 5-6 trucks per day each holding 38 tons of compacted garbage. The worker also ran the excavator, which was used to crush and load wood. He worked six days per week for the employer and did no renovation work or any other work while he was employed there. His duties also included maintenance work, greasing the machines, and cleaning up garbage that fell at the rear of the compactor, where it would get cleaned out by shovel so that it could be picked up by the loader.

[25] The worker testified that he had no prior problems with his right shoulder when he was hired in July 2012.

[26] Within a month of being hired, the worker was promoted to foreman/machine operator, indicating that he was a working foreman. According to the worker this was all an attempt to upset the injured employee with longer tenure, that the employer was attempting to get rid of. In addition to his loader/excavator duties, the worker was now required to oversee and assign work to the other workers and direct the trucks in and out of the transfer station.

[27] On February 18, 2013, at approximately 4:30 a.m., after a second truck had been loaded, the compactor shut down. As he walked around to the back of the compactor to reset it, he slipped on ice and fell. He landed onto his right elbow and right hip and felt immediate pain in his right shoulder and right hip. One of the truck drivers asked him if he was alright. He couldn't move his shoulder up; it was impossible. He felt pain over the top of the shoulder and the pain increased when lifting his arm. After that he didn't raise his right arm over his head. As no one else was around the worker asked the truck driver to write down a statement regarding what he had witnessed.

[28] The accident was reported immediately by the worker to his supervisor, J.B. when he arrived at work. The worker stated that the employer took the report and that A.B., the district manager, asked him not to file a WSIB claim, stating that the employer would take care of him. When the employer asked him how he felt, the worker indicated that he was in a lot of pain and that he couldn't move his right arm up and hoped that it was a strain that would get better in the next couple of weeks. The worker didn't want to lose his job; he loved his job, and had pride in what he was doing and so he agreed to not file a WSIB claim.

[29] After the accident, the worker self-modified his duties; he continued to run the loader and excavator, which he indicated only involved wrist/hand movements with respect to the upper extremities. He didn't run the excavator much, because you have to pull yourself up and he would get the other workers to run it. The loader was easier to get into because it had steps and hand rails. Once he was on the loader, he generally remained there all day. After the injury he no longer did the picking up of garbage and machine maintenance.

[30] On March 20, 2013, the worker sought medical attention for the first time because he realized that the right shoulder was not getting better, not even a little bit. He did not attend at his family doctor's office because his work was out of town and he stayed locally during the week returning home on weekends.

[31] Between February 18 and March 20, 2013 he took over the counter medication such as Tylenol extra strength, 2-3 in the morning, 2-3 at noon and 2-3 at night. With respect to a new claim for an incident dated May 24, 2013, the worker was referred to the Form 7. He said that the Form 7 was inaccurate and that there was a piece of metal (angle iron) in the way of the loader; it was not too big but it could damage the machine. He pulled it out and strained his right arm.

[32] At some point the worker discussed work modification with J.B. and A.B.; he was unclear as to exactly when this took place (before or after the diagnosis of a tear). They advised him not to do any physical labour after the injury and to let the other guys do it.

[33] The worker was referred to a statement from a co-worker dated June 20, 2013. The statement states the following:

I [A.A.] am a [employer's name] Operator working at [employer's address]. I have been working with [the worker] for approximately 4 months.

My statement is being provided to [J.S.] Corporate Risk and Wellness Manager, [A.S.] RTW /WSIB Coordinator and [A.B.] Operations Manager.

I have personally seen [the worker] lift heavy objects, specifically couches over his head and onto the top of his SUV vehicle over the last several months. I have also seen him lift trailers and hoist them onto his car's trailer hitch.

[34] The worker responded that this statement was a fabrication. He was referred to the ARO decision which stated that , "... He denied lifting a couch over his head stating instead that he used a machine to load the couch onto his SUV." The worker testified that a labourer put the couch onto the bucket of the loader and also put it into a utility trailer. The worker had various things in this utility trailer and he took them home. The couch was for a friend who unloaded the couch for himself. The remainder of the items were left on the trailer as they were outdoor articles.

[35] The worker testified that he continued his work as a foreman until his surgery in August 2013.

[36] At the time of the right shoulder surgery in August 2013 the worker went on Employment Insurance (EI) sick benefits. He returned to work after physiotherapy treatment and was terminated by the employer within two days for scavenging. The worker stated that when he was hired, he had an agreement with A.B. that he could take garbage home. When the worker returned to work in January 2014, employees were no longer allowed to take garbage off of the premises. He was terminated for retrieving a pair of work boots from the garbage and placing them in his vehicle.

[37] The worker's family doctor, Dr. Harmathy has been the worker's general practitioner (GP) for approximately 30 years. The worker rarely attends at the doctor's office. He did attend post-surgery for pain medication. He doesn't recall the last time that he saw his GP.

(vi) Submissions

[38] Ms. Robinson of the OWA, stated that the employer submits that they were unaware that the worker had ongoing pain as a result of the February 18, 2013 slip and fall and the ARO has accepted that evidence. The worker continued to work post-accident but only performed the essential duties of his job. He was asked not to file the claim and believed that the right shoulder

would recover on its own. When the four weeks had passed with no improvement the worker sought medical attention.

[39] Prior to the accident the worker had no problems with his shoulder. He reported the accident on the same date, continued to work using Tylenol, as he was reluctant to file a claim. All of this is confirmed in the worker's discussion with the Eligibility Adjudicator on April 22, 2013 noted in Board Memo # 3, which states:

While at work on Feb. 18th, 2013, he slipped and fell, he came down on his right elbow, with all his weight, which caused him to push his elbow/arm up into his shoulder.

He mentioned the incident the same day to [J.B.], however, he just thought that it was a strain and did not want to file a claim, he had only been with the company for 6 months and did not want to lose his job.

The driver also saw the incident when it happened, he filled out a statement on his behalf and gave it to the company (he does not know the driver's name).

He went to the MD on March 20th, 2013, because pain did not go away, he did not (sic) x-ray, but it did not show any issues with the bone, however, an ultrasound was done confirming a tear in his right shoulder.

There is no issue with his elbow or arm, it's his shoulder as his weight caused his arm to push up in the shoulder.

He kept working and doing his regular work, which he continues to do.

He works on running the machine, which requires the use of his wrist, not his shoulder.

For the month delay in seeing the MD, he complained about the ongoing pain in his shoulder to [J.B.] and Manager, [A.] (he does not know his last name).

He also took Tylenol to help cope with the pain.

He does home renovations on the side (he is paid cash and does not file taxes), but since the incident, he has not been able to do this work and this has caused a serious financial burden.

The last time he did home renovations was prior to the accident, he has not been able to do any work since.

He did not have any issues with his right shoulder, prior to his incident.

...

[40] Post-accident the worker's job duties were scaled back, no picking and no maintenance and he could continue working as long as he did not exceed his shoulder restrictions. As an example, she referenced the incident of May 24, 2013, when the worker tried to remove metal pieces from the ground, resulting in a flare up his right shoulder symptoms.

[41] The worker's representative referenced a letter from the employer to the WSIB dated June 20, 2013 asking for a reconsideration of the allowance of the rotator cuff strain under the May 24, 2013 incident arguing that no new injury occurred and that the worker had already been diagnosed with a rotator cuff tear. The representative agreed that there was no new accident but that the symptoms were consistent with the worker's original February 18, 2013 accident.

[42] Ms. Robinson referred to written submissions from the OWA on file dated July 28, 2014 and to the Tribunal's Medical Discussion Paper, *Shoulder Injury and Disability*, stating that if there had been any pre-existing tear in the shoulder it had been asymptomatic prior to the accident. According to the representative, the mechanics of the accident are compatible with the

rotator cuff tear; the self-accommodation speaks to a post-injury change in the worker's duties, and the delay in seeking medical attention was reasonable given the worker's initial belief that it was a sprain that would resolve. The employer's attitude towards work injuries and the worker's fear of losing his job were also factors in the worker's reluctance to seek medical attention. Given all of the evidence, the delay in seeking medical attention was reasonable and initial entitlement should be allowed.

(vii) Analysis

[43] The Panel Majority concludes the appeal is allowed for the reasons set out below.

[44] OPM Document No. 11-01-01, "Adjudicative Process", states that an allowable claim must have five points: an employer, a worker, personal work-related injury, proof of accident, and compatibility of diagnosis to accident history. OPM Document No. 11-01-01 provides the following guidelines for determining proof of accident:

Proof of accident

Decision-makers may consider the following when examining proof of accident,

Does an accident or disablement situation exist?

Are there any witnesses?

Are there discrepancies in the date of accident and the date the worker stopped working?

Was there any delay in the onset of symptoms or in seeking health care attention?

(a) Did an accident occur on February 18, 2013 and did the worker suffer from any pre-existing condition?

[45] In this case the Panel Majority must determine whether or not the worker sustained a slip and fall accident on ice on February 18, 2013, and whether this accident resulted in a personal injury, namely a right shoulder injury/rotator cuff tear, that required surgery on August 13, 2013. In making this determination, the Panel Majority must also consider the onset and continuation of the worker's symptoms, the reasons for the worker's delay in seeking medical attention and compatibility between the accident and the right shoulder injury/rotator cuff tear.

[46] Firstly, the Panel Majority accepts that the worker fell at work on February 18, 2013, and that he immediately reported the accident and injury to his right shoulder to his employer. The worker's testimony in this regard is corroborated by the employer's Form 7 and the witness statement submitted by the worker. The employer has never argued that no accident was reported.

[47] With respect to the worker's pre-injury right shoulder condition, the Panel Majority finds that there is no evidence of significance to suggest that the worker had any problems with his right shoulder prior to the February 18, 2013 accident. In coming to this conclusion the Panel Majority notes that none of the medical evidence on file references any pre-existing problems with the worker's right shoulder. We also note that in the Form 7, dated March 28, 2013, the employer indicates that the area of injury was the worker's right shoulder and right arm and that they were unaware of any prior similar or related problem, injury or condition. Further, there is no other subsequent evidence provided by the employer indicating that the worker had any pre-existing problems with his right shoulder. Finally, we further note and accept the worker's

testimony to the effect that he had no problems with his right shoulder prior to the slip and fall on ice on February 18, 2013.

(b) Onset of symptoms, continuity of complaint and delay in seeking medical attention

(1) Employer's evidence

[48] After receiving the Employer's Report of Injury dated March 28, 2013, the Eligibility Adjudicator (EA) contacted the employer on April 19, 2013. The employer's Human Resources (HR) Associate informed the EA that the worker did not mention any problems after he fell and that he continued performing his regular duties. The worker now advised them that he required surgery for a right shoulder tear. The worker was placed on modified work, limiting any activity that affected the right shoulder.

[49] After speaking to the worker on April 22, 2013, the EA contacted the worker's Supervisor, J.B. as well as A.B., the District/Operations Manager on April 23, 2013. J.B. stated the following:

The worker advised him that he slipped on Feb. 18th, 2013, but he said that he was fine and that he was not injured. The worker did not mention anything to him about being injured or having an issue with his shoulder until about 5-6 wks after when he told him that he had to have surgery. He was not aware of any ongoing issues until the worker mentioned having surgery about 5-6wks after. The worker continued to do his regular work without issues.

[50] The District Manager stated:

The worker never reported injuring himself at work to him.

He has known the worker for a long time and once (he does not remember the date), he saw the worker stretching, he asked the worker if he was okay, the worker mentioned that he slipped awhile back and was a bit tight, but that he was fine.

The worker did not complain/mention any issues with his shoulder to him.

[51] The Panel Majority notes that this evidence from the worker's Supervisor and the District Manager is at odds with the worker's recollection of events. The significance of the disparity between the worker's evidence and the employer's evidence on any continuity of complaint and compatibility will be discussed in greater detail below.

[52] On May 28, 2013, the employer's Return to Work (RTW) Coordinator, filed a new claim for the worker with respect to a May 24, 2013 incident that caused increased pain in the worker's right shoulder, objecting to any compatibility of any injury as the worker was already symptomatic and awaiting surgery and since no specific incident had occurred.

[53] On June 20, 2013 the employer wrote a letter to the WSIB with respect to the allowance to health care benefits in relation to the May 24, 2013 right shoulder strain. In that letter the employer's Risk and Wellness Manager wrote the following:

...Given the history of a rotator cuff strain his claim should not have been allowed without significant medical information confirming a new area of injury. Per our discussion with [the worker] in a meeting today, June 20, 2013 [the worker's] pain following the alleged May 24, 2013 incident remained in the same area of his pre-existing rotator cuff tear. [The worker] further explained that he was lifting small pieces of metal with his left hand and using his right arm to assist, when he experienced pain in the right shoulder. [The worker] did not seek immediate medical attention for his

strain and only went to see his family doctor to retrieve a medical note related to his prior claim and happened to mention he was in pain. [The employer] did not receive a new form 8 or a functional abilities form relative to this alleged May 24, 2013 incident. We received a physical abilities form for a non work related condition that indicated he was booked for repair of a right rotator cuff tear of August 13, 2013.

As discussed , we submit that [the worker] went to his family doctor to discuss his ongoing pain related to his pre-existing rotator cuff tear. Furthermore, since the May 24, 2013 incident [the worker] has been able to continue on with his duties. We also submit that he hasn't deteriorated as has been seen on surveillance camera lifting couches over his head and onto his SUV vehicle. According to our surveillance camera this occurred on June 13, 2013. In reviewing this matter we also obtained a signed statement from [the worker's] co-worker confirming that he has seen [the worker] numerous times over the last several months lifting couches and heavy objects onto his SUV. This also included heavy trailers.

...

[54] The Panel Majority notes and accepts that at least as of the date of the employer's Form 7 Report dated March 28, 2013, the employer was notified that the worker had sustained a rotator cuff tear that required surgery since the employer referred to this condition and the surgery that was required in its form. In its June 20, 2013 letter, the employer refers to the worker's activities at work on June 13, 2013, that indicated that the worker exceeded his right shoulder restrictions, corroborated by video surveillance on that date, which video surveillance evidence is not contained in the file materials for this appeal. The purpose of their letter was to object to the allowance of the May 24, 2013 claim on the basis of the worker's pre-existing non-compensable right rotator cuff tear. The employer did not argue that any of the other activities that the worker was engaged in on its premises caused the tear in the worker's right shoulder. The employer ascribes the worker's increased right shoulder pain on May 24th to his pre-existing tear.

[55] The employer's letter also references a signed statement from a co-worker who witnessed the worker performing activities at work that would have exceeded his shoulder restrictions. This statement, from A.A. is dated June 20, 2013, contemporaneous to the employer's objection letter and states:

I [A.A.] am a [Employer] Environmental Operator working at [employer's address] I have been working with [the worker] for approximately 4 months.

My statement is being provided to [J.M] Corporate Risk and Wellness Manager, [A.S.] RTW /WSIB Coordinator and [A.B.] Operations Manager.

I have personally seen [the worker] lift heavy objects, specifically couches over his head and onto the top of his SUV vehicle over the last several months. I have also seen him lift trailers and hoist them onto his car's trailer hitch.

[56] We also note that this co-worker, A.A., refers to various activities performed by the worker at his place of employment but he does not state that any of these activities were more likely the cause of the worker's right shoulder tear nor did this co-worker provide any evidence that contradicted the worker's claim that he had sustained a right rotator cuff tear as a result of the slip and fall in February 2013.

[57] Accordingly, the Panel Majority cannot ascribe any significant weight to the employer's objection letter nor to the co-worker's statement, as providing proof that the worker's right shoulder tear is non-compensable. Even if the employer was unaware of any ongoing problem until it filed the Form 7 on March 28, 2013, we have no evidence of substance either from the

employer, co-workers or medically, supporting any intervening event that would break the chain of causation between the slip and fall accident that certainly occurred and in which the worker immediately claimed that he injured his right shoulder on February 18, 2013 and the diagnosis of a rotator cuff tear in March 2013. Further, based on the aforementioned reasons, even if the worker had participated in activities which exceeded his right shoulder restrictions in the course of his employment, the Panel Majority finds that those activities would not negate any causal connection between the fall and the right shoulder tear as we have no evidence to substantiate an alternative cause and in any event we accept the worker's testimony that the pain and inability to lift the right shoulder commenced with the accident on February 18, 2013 and was ongoing from that time. It is therefore the Panel Majority's finding based on the balance of probabilities that the right shoulder tear occurred during the workplace accident on February 18, 2013.

(2) Worker's evidence

[58] In addition to accepting the worker's testimony that he experienced right shoulder pain and an inability to lift the right shoulder from the time of the February 18, 2013 accident, the Panel Majority also accepts his testimony that he initially hoped that the right shoulder injury was merely a sprain, that he was concerned about losing his job and that he delayed in seeking medical attention because he expected it to resolve. The worker has remained consistent in his explanation for the delay in his Form 6 dated May 27, 2013, in his conversation with the EA on April 22, 2013 and in his testimony at the hearing. We accept the worker's testimony that he was concerned about the loss of his job and that he self-accommodated with respect to his work duties. We accept that he was able to continue to run the loader as he had reported to Dr. Dworak on March 20, 2013 and remain at work with the assistance of over the counter medication. We accept that the worker experienced onset of symptoms immediately following the accident and that these symptoms continued.

[59] Based on the foregoing reasons, the Panel Majority finds that the four week delay in seeking medical attention was reasonable and not excessive (See *Decision No. 1350/19*). As noted above the worker had no history of any prior right shoulder problems and there is no evidence from the employer, co-workers or health care practitioner of any intervening event. We therefore find, that the worker's reason for seeking medical attention on March 20, 2013 for the right shoulder was directly related to the accident of February 18, 2013.

(c) Compatibility between the accident of February 18, 2013 and the right shoulder tear

[60] The Panel Majority observes that the accident history provided by the worker has always remained consistent in the worker's reporting to Dr. Dworak on March 20, 2013, to his family doctor, Dr. Harmathy on May 27, 2013 and to his orthopaedic surgeon, Dr. Randle on May 31, 2013. Dr. Dworak filed a Form 8 with a diagnosis of a torn right shoulder on March 20, 2013 subsequent to a fall at work onto the shoulder on February 18, 2013. The full supraspinatus tear was confirmed through an ultrasound performed on March 27, 2013.

[61] The worker attended at Dr. Harmathy's office on May 27, 2013, advising the family doctor of a fall on ice and landing on his right shoulder, resulting in a "...total thickness tear of the supraspinatus muscle involving the shoulder."

[62] Dr. Randle stated in his initial report to Dr. Dworak of May 31, 2013 that the worker "...fell on ice suffering a rotator cuff tear of his right shoulder." An arthroscopic rotator cuff reconstruction was recommended with surgery taking place on August 13, 2013.

[63] The Panel Majority places significant weight on the consistency of the worker's reporting of the accident history to his health care practitioners. We find that the consistent recording of the mechanics of the injury and the resulting rotator cuff tear by the worker's health care practitioners, without any reference to any other cause, implicitly supports a causal connection between the fall and the tear. We also interpret the reporting from the treating health care practitioners, including from the orthopaedic surgeon Dr. Randle, to mean that these physicians were likely of the view that the worker's right shoulder diagnosis of a rotator cuff tear was compatible with the accident history. As no evidence of an alternate explanation or intervening event exists that would break the chain of causation, we find on a balance of probabilities that the fall on ice was the cause of the rotator cuff tear.

[64] According to the Tribunal's Medical Discussion Paper on shoulder injuries, rotator cuff tendons may tear in response to a severe injury where the external force is greater than the strength of the healthy tendon, as in young individuals. In older individuals the tear can occur with a moderate external force, such as when lifting objects. At the time of his injury the worker was approximately 52 years old. The Panel Majority is satisfied that the worker's fall on ice on February 18, 2013 as an older worker constituted, at the very least, a moderate external force that resulted in the worker's right rotator cuff tear.

[65] On the basis of the foregoing reasons, the Panel Majority concludes that proof of accident has been established and that the worker's right shoulder injury/ rotator cuff tear is medically compatible with the fall on ice on February 18, 2013.

(d) Conclusion

[66] The Panel Majority finds that the preponderance of the evidence supports that the worker sustained a slip and fall on ice while at work in the course of his employment on February 18, 2013 that resulted in a right shoulder injury/rotator cuff tear and a requirement for surgery on August 13, 2013. The evidence indicates that the repair surgery which occurred on August 13, 2013 was necessary and required to treat the right rotator cuff tear. Therefore, initial entitlement for a right shoulder injury and the required surgery of August 13, 2013 is allowed.

DISPOSITION

[67] The Panel Majority has determined that the appeal is allowed as follows:

1. Initial entitlement for an accident on February 18, 2013, resulting in a right shoulder injury/rotator cuff tear is allowed.
2. Entitlement to benefits for the right rotator cuff surgery on August 13, 2013 is allowed.

[68] The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: May 29, 2020

SIGNED: C. Sacco, M. Ferrari



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 667/19

DISSENT

REASONS

(i) Introduction

[69] The worker appeals a decision of ARO Sheridan, dated January 19, 2016, which denied initial entitlement for a right arm and shoulder injury on the basis that proof of accident was not established. The ARO rendered a decision following an oral hearing held on January 13, 2016.

(ii) Background

[70] This now 57 year-old Machine Operator was hired by his accident employer in July 2012. On February 18, 2013, he reported slipping and falling on ice and injuring his right shoulder. He did not seek medical attention until March 20, 2013. He was sent for an ultrasound and x-rays. He continued working until May 27, 2013, when he submitted a Form 6 report of injury. On August 13, 2013, he underwent a right shoulder arthroscopy, synovectomy and rotator cuff reconstruction.

[71] In a decision dated April 30, 2013, the Eligibility Adjudicator denied entitlement on the basis that there was no continuity of complaint to show that the worker's right shoulder condition on March 20, 2013 was related to his February 18, 2013 slip-and-fall incident.

[72] In a reconsideration decision dated September 30, 2014 the Eligibility Adjudicator upheld the denial of entitlement, finding that new evidence submitted to file did not establish that the worker was symptomatic while working between his slip-and-fall and seeking medical attention.

[73] The worker objected to the denial of initial entitlement and an oral hearing was held on January 13, 2016. In a decision dated January 19, 2016, ARO Sheridan upheld denial of initial entitlement for a right arm and shoulder injury on the basis that proof of accident was not established.

[74] The worker now appeals to the Tribunal from this decision.

(iii) Issues

[75] The sole issue under appeal is initial entitlement for a right shoulder and right arm injury as arising out of and in the course of the worker's employment.

(iv) Law and policy

[76] Since the worker claimed to be injured in 2013, the *Workplace Safety and Insurance Act*, 1997 (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[77] An “accident” is defined in section 2(1) to include:

- (d) a wilful and intentional act, not being the act of the worker,
- (e) a chance event occasioned by a physical or natural cause, and
- (f) disablement arising out of and in the course of employment.

[78] General entitlement to benefits is governed by section 13:

13(1) A worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan.

(2) If the accident arises out of the worker’s employment, it is presumed to have occurred in the course of the employment unless the contrary is shown. If it occurs in the course of the worker’s employment, it is presumed to have arisen out of the employment unless the contrary is shown.

[79] The statutory presumption set out in section 13(2) does not apply to an injury by disablement. See, for example, *Decision Nos. 268 and 42/89*.

[80] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

[81] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision #9, would apply to the subject matter of this appeal:

- Package #241 – Initial Entitlement
- Package #300 – Decision Making / Benefit of Doubt / Merits and Justice.

[82] I have considered these policies as necessary in deciding the issues in this appeal. In particular *Operational Policy Manual (OPM) Document No. 11-01-01, “Adjudicative Process”*, states that an allowable claim must have five points: an employer, a worker, personal work-related injury, proof of accident, and compatibility of diagnosis to accident history. OPM Document No. 11-01-01 provides the following guidelines for determining proof of accident:

Proof of accident

Decision-makers may consider the following when examining proof of accident,

- Does an accident or disablement situation exist?
- Are there any witnesses?
- Are there discrepancies in the date of accident and the date the worker stopped working?
- Was there any delay in the onset of symptoms or in seeking health care attention?

(v) Analysis

[83] After due consideration, I am regretfully unable to concur with my colleagues as to the outcome of this appeal. I would deny the appeal for the reasons set out below.

[84] Following the hearing, the Panel requested the clinical records of Dr. Harmathy, the worker's family doctor, for five years prior to February 18, 2013 and until the date of the worker's surgery. These materials were obtained and provided to the worker's representative, who then provided written submissions on the merits dated October 2, 2019. I have considered these further submissions in addition to the evidence and submissions provided at the hearing.

(a) The February 18, 2013 incident

[85] Both the employer and worker have indicated that the worker reported slipping and falling on ice while working on February 18, 2013. This occurrence is not in dispute. What is at issue is whether the worker's rotator cuff tear, identified by ultrasound just over a month later, was related to this incident.

[86] In an April 22, 2013 telephone conversation with the Board, the worker stated that on February 18, 2013, he had fallen on his right elbow, pushing up on his shoulder. At the present hearing, the worker confirmed this accident history, indicating that when he slipped and fell, he landed on his right elbow and hip, not his shoulder, but felt pain in his right shoulder and hip.

[87] I have carefully reviewed and transcribed the worker's testimony from the hearing recording. The worker testified that he was in working before any other employees, to load three trucks by the time the scale operator arrived at 6am. While loading the second truck, the compactor shut down. He left the loader and went outside the building to reset the compactor. That's where he fell. When asked to describe how he fell he stated [27:45]:

Well, my feet went out. I went down on my elbow and my hip, on the ice. And my shoulder went up, and it pulled out, I guess it pulled out, and it ripped the muscle where it connects to the bone.

[Chair:] So you knew that at the time? Did you know that at the time?

I didn't know that at the time.

[Chair:] Ok, so what we're getting at is only what you experienced at the time.

Um, yes. I fell and hurt myself.

[Chair:] Ok, so you landed on your elbow and hip??

Yes

[Chair:] Ok, and what did you feel at the time?

Pain

[Chair:] And where was the pain?

In my shoulder and my hip, like, my bum.

[88] He demonstrated how he could not lift his right arm above his shoulder as a result of this incident. He did not describe problems with the elbow on which he fell. He was then asked what he felt when he tried to lift his shoulder. He indicated that he had pain at the top of the shoulder, not in the shoulder joint or in the shoulder blade. He then said that he never raised his arm over his head after that.

[89] Considering this testimony, I note that the worker initially skipped from an account of what he experienced at the time to what he was later told by the specialist Dr. Randall (as he confirmed in later testimony). The worker appeared to want to give the impression that he knew he had suffered a severe injury on February 18, 2013. But he contradicted this when he stated,

consistent with his May 27, 2013 Form 6 that he thought at the time that he had only suffered a sprain. This was not the only the time the worker contradicted his own testimony in the hearing.

[90] When I consider what he said he actually experienced at the time, he did not repeat anything about his shoulder being dislocated or ‘pulled out’. He described feeling pain in his shoulder, but also on ‘his bum’ where he landed. The worker described having pain in his hip as a result of his fall, but not in his elbow. This implies that he fell harder on his hip/buttocks than on his elbow. This further implies that the worker’s hip and buttocks took the greater force of the impact.

[91] The elbow does not hang lower than the hip when a person is upright. If the worker landed on his ‘bum’ or even on his right hip in an upright position, his elbow would not have hit the ground. The worker did not describe any elbow pain or injury as one would expect from a direct blow to the elbow tip, but he was definite that his elbow also hit the ground when he fell. This means that the worker must have been in a semi-reclined posture when he hit the ground, in order to have made contact (even sequentially) at the two points he describes. Hitting the ground in this posture, his elbow would have been driven forward or inward rather than upward. The injury mechanics as the worker described them would not have jarred his elbow upwards into the shoulder, but forward or inward. It would have been more likely to create a bruise on the elbow and hip than on the shoulder, which did not strike the ground, according to the worker’s own description of the February 18, 2013 incident.

(b) First medical assessment

[92] I find that the worker’s description of the February 18, 2013 incident is anatomically incompatible with the right shoulder injury seen by Dr. Dworak on March 20, 2013.

[93] Dr. Dworak’s Patient Encounter Worksheet of 2:10pm on March 20, 2013 states that worker was a walk-in patient and records that the worker fell on his right shoulder two weeks ago and can’t lift his arm. Accepting this history would provide an injury date in early March, rather than mid-February. In addition, the worker, in his own reporting and testimony, did not describe falling onto his shoulder, but rather onto his elbow, on February 18, 2013. Dr. Dworak does not record the fall onto the shoulder in the Patient Encounter Worksheet as happening at work, which raises a possibility of two different incidents.

[94] I accept the initial Patient Encounter Worksheet evidence at its face value. No adequate explanation has been presented, to my mind, as to why Dr. Dworak would have recorded the worker falling onto his shoulder in early March if in fact he did not do so. The occurrence of another falling incident in early March 2013 is the most plausible explanation of why the worker did not seek medical attention earlier.

[95] However, Dr. Dworak and the worker also completed a Form 8 report which was faxed to the Board at 4:15pm on the same day. This second document specifies February 18, 2013 as the date of injury. The March 20, 2013 Form 8 does not provide physical findings, but provides a primary diagnosis of a contusion/hematoma/swelling, suggesting that there was a visible mark or swelling at the time of the March 20, 2013 examination. Dr. Dworak does not specify where this visible mark is located, but the report mentions only the worker’s shoulder and makes no reference to the worker’s elbow. I find, on a balance of probabilities, that the contusion/hematoma/swelling that Dr. Dworak noticed was on the worker’s shoulder, not the elbow on which he had reportedly fallen on February 18, 2013. I also note that the appearance of

such a mark appears to be consistent with a more recent injury than one suffered a month earlier. No adequate explanation of how the bruised shoulder Dr. Dworak saw on March 20, 2013 could have been produced by the February 18, 2013 incident has been provided.

[96] There are two handwritings on the Form 8. The form is signed by the worker. The Form 8 states that the worker can try regular duties. No limitations are indicated. The boxes for 'Use of upper extremity' and 'push/pull' are not ticked. Under this are two notations which are hard to decipher and appear to be misspelled. The first appears to say 'continue duties'. The second appears to say 'climbing [sic] into machi[n]e most [gate?] problem'. Close to the portion that has the notation 'Feb 18 Fall onto R shoulder' the worker's job is spelled 'opperator' as it is on the worker's Form 6.

[97] I find that the injury on the Form 8 is "Contused or torn shoulder", the word 'or' being written in the same fashion as the 'or' in 'torn'. In any case, there is definite evidence that the shoulder was contused. I find that this is consistent only with a direct trauma to the shoulder.

[98] There is no evidence that the worker contused his shoulder or fell on his shoulder on February 18, 2013. Therefore, the preponderance of written evidence, interpreted reasonably and in accordance with its plain meaning indicates that the injuring event was not the same as the February 18, 2013 incident. I fail to understand how the worker's shoulder contusion could reasonably be attributed to a fall in which the worker did not strike his shoulder, more than a month previous. Bridging this critical gap in the evidence requires speculation which I am unwilling to undertake.

[99] My colleagues do not find the discrepancy in the initial reporting to be significant, whereas I do. I believe that the appearance of the Form 8 indicates that it was jointly authored and is not the sole creation of Dr. Dworak. I therefore find that the information appearing on the earlier Patient Encounter Worksheet is more reliable than that on the Form 8.

[100] I would find, on a balance of probabilities, that the accident history provided to Dr. Dworak on March 20, 2013 is not consistent with the work incident report of February 18, 2013. I also note that the witness report provided by a driver not employed by the accident employer, while mentioning that the worker slipped and fell on February 18, 2013, does not say anything about the worker injuring his right shoulder.

[101] The worker was sent for an ultrasound on March 22, 2013 which revealed a frontal tear of the supraspinatus tendon with some bursal fluid and hemorrhagic biceps tenosynovitis. It is instructive to compare this with what was recorded in Dr. Randle's operative report of August 13, 2013. This described the condition surgically repaired as a massive tear of supraspinatus and infraspinatus tendons with retraction beyond the level of the glenoid. The tear was described as a transtendinous tear as opposed to an avulsion type tear. The degree of retraction of the torn tendons meant that they could not be reattached to the greater tuberosity. Instead, the biceps tendon was laid across the greater tuberosity to function as a vascularized graft. Dr. Randle summarized that the condition of the tissue observed was relatively poor, but was augmented nicely with the biceps tendon.

[102] Prior to the present hearing, a Tribunal Medical Discussion Paper entitled "Shoulder Injury and Disability" by orthopaedic surgeon Dr. Hans Uthoff (Revised October 2010) was provided for reference by the Panel and parties. In the present case, I would note and adopt the following statements appearing at page 14 of the Discussion Paper:

Rotator cuff tendons may tear in response to a severe injury where the external force is greater than the strength of a healthy tendon (young individuals). In this instance, a piece of bone is often avulsed (torn away by force) from the greater tuberosity of the humeral head together with the tendon. The greater and the lesser tuberosities are bony prominences of the humeral head; they are situated outside the glenohumeral joint (extraarticular)

In the more common type of rotator cuff tear, the tearing (Figure 7) occurs in older individuals with a moderate external force, such as during the lifting of objects. It causes a tear of a cuff tendon close to its insertion into bone. This site has been weakened by age-related, and maybe by activity-related, changes (middle aged and older persons), already present at the time of the incident. The incidence of age-related rotator cuff tears increases with age to a point where tears may occur in the absence of a history of injury. This happening must be considered as a disorder and not as an injury. Following a tear, active movements are restricted or impossible since the continuity between origin and insertion of the muscle-tendon-bone unit is disrupted. As the supraspinatus tendon is usually affected, it results in impaired abduction. The person cannot hold his/her arm in a position at 90 degrees away from the body (abduction). If the infraspinatus is torn, external rotation is weak and if the subscapularis is disrupted, internal rotation is affected.

[103] I note that the worker's massive tear of supraspinatus and infraspinatus tendons was not an avulsion type tear, as is often related to severe injuries caused by external trauma. I accept Dr. Randle's operative observation that the tendon tissues were of poor quality. I also accept that a retracted full tearing of these two tendons could be expected to be accompanied by a significant loss of function when holding the arm away from the body or rotating it externally, which the worker did not describe.

[104] I find that there is no objective evidence that the worker experienced these functional restrictions in relation to his February 18, 2013 incident, or that he sought medical attention in keeping with such significant tendon tearing at or near the time of this work incident. The first record of significant right shoulder restrictions on abduction and external rotation in Dr. Harmathy's clinical records appears on May 27, 2013, some two months after the ultrasound finding of a full thickness supraspinatus tear.

[105] If we are to believe that the damage that is eventually recorded in the August 2013 operative report happened on February 18, 2013, he suffered retracted tears to both his supraspinatus and infraspinatus rotator cuff tendons, as documented in that report. However, the ultrasound of March 22, 2013 identified only a supraspinatus tear and found that the other tendons were intact. The worker has denied having another injury. This suggests that the infraspinatus tear developed later without trauma.

[106] I also note that the worker was seen on April 24, 2013 by the orthopaedic specialist, Dr. Randle. His report indicated that the worker had a torn tendon and stated:

Using a loader. O[n] e[xamination] decreased ROM of right shoulder Diagnosis: tendon tear, tendinitis.

(c) The family doctor's evidence

[107] Dr. Harmathy's records do not indicate that the worker reported or suffered from any right shoulder symptoms at the time of his February 18, 2013 slip-and-fall incident or at any time prior to his first seeking medical attention for his right shoulder on March 20, 2013. The doctor confirms that the worker did not seek medical attention from him between February 18, 2013 and March 20, 2013. Nevertheless, the worker testified that he drove to and from work every day as

well as to his permanent residence in the country, every weekend. During this time he never went to a hospital or walk-in clinic.

[108] The May 27, 2013 clinical note of Dr. Harmathy reads as follows:

He has slipped on some ice and landed on his right shoulder. He was seen at another hospital and x-rays were normal however he has a total thickness tear of the supraspinatus muscle involving the shoulder: He is unable to abduct and externally rotate. This limits his ability to work.

[109] Both Dr. Harmathy and Dr. Dworak understand that the worker fell 'on his shoulder'. Once again, interpreting the written evidence according to its normal meaning indicates that the history recorded by Dr. Harmathy is not consistent with the February 18, 2013 fall.

[110] I do not place greater weight on whether the worker said something about surgery to his employer by March 28, 2013. I find that there is no evidence that any such statement was related to any event prior to March 6, 2013 (two weeks prior to the March 20, 2013 first treatment). There is no evidence that any examining doctor discussed surgery until after the ultrasound of March 22 was reviewed, nor does this seem likely. I also note that the Employer's Form 7 report completed on March 28, 2013 indicates that the worker said he did not require medical attention after the February 18, 2013 slip-and-fall, and continued to perform his regular duties including overtime. This would support the same conclusion.

[111] The May 31, 2013 report of the orthopaedic surgeon Dr. Randle does not mention a February injury date for the fall that caused the right shoulder supraspinatus tear. I note that Dr. Randall does not record any indication of an infraspinatus tear at the time. His report states:

Thank you for referring [the worker] to see me. As you know, he is a 52-year-old machine operator who has a history of right shoulder pain. He fell on ice suffering a rotator cuff tear of his right shoulder.

On examination, he has half normal active range of motion with ostensibly full passive range of motion. He has weakness with testing of his supraspinatus.

Imaging confirms a full thickness tear of his supraspinatus.

IMPRESSION AND PLAN: [the worker] would benefit from an arthroscopic rotator cuff reconstruction. The risks and benefits of which were explained to him. I believe he understood and consent was obtained. We will be proceeding in the near future.

[112] This is the first recommendation of surgery from a health care professional in the file.

[113] My colleagues have accepted the worker's testimony that he could not recall any other injury (not even the incident that led to the filing of a second claim on May 27, 2013) and delayed reporting because he thought his injury was just a sprain that would get better. I find that there are several factors which throw the veracity and reliability of the worker's testimony into doubt. There is also evidence that the worker's shoulder function was better than he indicated in his testimony.

(d) The worker's functional abilities after February 18, 2013

[114] The worker testified that he had previously worked with heavy equipment such as loaders and excavators, and had been made a foreman with the accident employer in September 2012 with the ability to assign tasks a direct other employees at his jobsite after that. After his injury, he was able to do his regular job, running both loaders and excavators, but noted difficulty getting in and out of a loader. He would use his left arm to pull himself up the ladder with two

handrails. He would operate an excavator to crush wood. He would also still operate the loaders which had a forward and reverse control operated with the right hand. The right hand controls were also used to control bucket operations. He continued to commute each day. He would take over-the-counter medication such as Extra Strength Tylenol, but had no other treatment.

[115] Contrary to the worker's testimony, according to a memo dated April 19, 2013, the Eligibility Adjudicator spoke with the employer and was told:

The worker did not mention anything to anyone after he initially fell, he continued to work his regular duties, then came into work advising that he needed surgery..

She has since placed the worker on modified work, where he will limit activities that affect his shoulder.

[116] On April 22, 2013, the Eligibility Adjudicator spoke with the worker and recorded him saying in part:

He kept working and doing his regular work, which he continues to do.

...

Right now, the shoulder does not prevent him from doing his regular work, however, after surgery he is afraid he will not be able to do the work.

[117] I note that the employer also indicates in a June 20, 2013 letter that they have surveillance footage showing the worker lifting a couch onto his SUV on June 13, 2013. Although I doubt the employer would say it had surveillance footage if it did not, such an incident has little bearing on the question of initial entitlement in this case, which hinges on what happened in February and March 2013 (by which time there is definite evidence of an injury). This information merely suggests that the worker's later operative findings had not fully developed at that time. Of more interest to me is evidence regarding the worker's use of a trailer.

[118] The employer submitted a signed statement of a co-worker, dated in June 2013, who stated he had worked with the worker for approximately four months (i.e. since late February). He stated in part:

I have personally seen [the worker] lift heavy objects, specifically couches over his head and onto the top of his SUV vehicle over the last several months. I have also seen him lift trailers and hoist them onto his car's trailer hitch.

[119] To my mind, the significance of this evidence is not with regard to an additional injuring process, as my colleagues appear to have thought. In fact no allegation of a further injury is mentioned in this evidence. Rather it is evidence that speaks to the worker's functional abilities after his February work incident, based on direct observation by a third party.

[120] In response, at the ARO hearing, the worker denied lifting a couch by hand, but stated he had loaded a couch onto his SUV, using a machine. At the present hearing, the worker again confirmed that he had loaded a couch onto his SUV using the loader at work. He also indicated that he would salvage 'a whole bunch of stuff' that he would collect and then load onto his trailer. He indicated that this activity was the reason he was ultimately fired in January 2014.

[121] When asked about the co-worker who provided the signed statement, the worker disagreed that he was there when his accident happened. When asked to review the statement about regarding lifting couches or trailer hitches, he was categorical, saying:

[1:09:50] No. No. That's a lie. That's a fabrication!

[122] He was then referred to his testimony at the ARO hearing [1:11:00]:

Stating that he had used a machine to load the couch onto the SUV?

Yes, and also another worker was helping. Yeah. I was running the machine. I was in the machine. It was my truck.

[Panel member: Was this at work?]

Yeah, this was at work. This was something that came out of the garbage.

[Panel member: You remember this specific incident?]

Yes, I do, yeah.

[Panel member: This was after your injury?]

Yes

[Panel member: And so what happened?]

I was in the machine... and the labourer put it into the bucket, and I put it on top of my truck, and he pulled it on top of my truck and then tied down. That's it.

[Panel member: How did you put it on top of your truck?]

With the loader. You lift it up, tilt it down and the labourer put it on...

Yes, it's an SUV. It had roof racks on it...

[123] I note that the worker thus confirmed many details of the statement he had previously called a fabrication and a lie, to be accurate: that he took a couch from work, that it was loaded on his SUV, that this took place in the four months after February 2013. I am unwilling to join my colleagues in accepting the worker's inconsistent testimony in favour of the roughly contemporaneous signed statement which the worker (only after being challenged) corroborated, in all respects except as to whether he participated in the actual physical loading and tying down (which likely would have been fatal to his claim).

[124] The standard of adjudication under the WSIA is proof on a balance of probabilities. In my understanding, this requires a critical assessment of contested evidence. I would note that in the present case, the worker initially could not remember when he started work for the accident employer – a date only about six months prior to his first medical attention for the injury he claims. The worker sought to impugn the evidence of a written and signed statement made no more than four months after the incidents it describes, by calling it a lie and a fabrication. Only when challenged by his own previous sworn statement did he (within a minute) purport to demonstrate a detailed recollection of the incident the witness statement described, the existence of which he had categorically denied. I find that this demonstrates that the worker's testimony is not reliable.

[125] Another reason for accepting the contemporaneous written statement over the worker's testimony is that the worker also retracted his denial of using a trailer [1:13:30]:

[Panel member 1: So, you said you don't know what [the author of the signed statement] is referring to where he says you were lifting trailers onto a trailer hitch?]

No.

[Panel member 2: Do you have a trailer?]

Yeah, I had it there. I had a whole bunch of stuff that I collected because I was allowed to collect it to offset the wage. And I had to clear it out of there because of the accident because I wasn't doing any more collecting, so everything had to go. So they helped me load up the trailer. I took it home. That was it.

[126] I find that this testimony relates this trailer incident to shortly after the worker's right shoulder problems started, because he remembers the incident happening 'because of the accident.' I find that this undermines the worker's earlier claim that the author of the signed statement was not working with him for four months prior to June 2013.

[127] I find that this further corroborates the signed statement. Once again, the worker denied knowing about using a trailer and only said that he knew exactly what the signed statement was referring to when challenged. Initially, the worker said that several people 'helped' him load up the trailer. Later he said that only one worker did the loading. He continued [1:15:50]:

He helped me load it up. That was it. The trailer never came back and I never did anything more.

[And where was the trailer taken?]

To my house.

[Who lives with you?]

Just me.

[So who unloaded the trailer?]

No-one. It was just left in the trailer. It was all outside stuff like bicycles and stuff like that. Yeah.

[Who unhitched the trailer?]

My girlfriend helped me do it. I had the use of my left hand and its not like a large trailer. It's a utility trailer.

[128] The worker had testified that his girlfriend lived in a different city from his house, and that he would only go to his house on the weekends.

[129] I accept the contemporaneous evidence and especially the statements to the Eligibility Adjudicator as indicating the worker's continued ability to work without notable restriction between February 18, 2013 and March 20, 2013. I prefer the more timely evidence documented in the file to the worker's testimony at hearing, and would find, on a balance of probabilities, that the worker was still using his right arm at work without notable restriction up to at least a short time before March or April, 2013.

[130] I note that tears of previously-weakened tendon tissues can occur in the absence of specific injury in middle-aged and older persons, and find that the worker falls into that category, having been 51 years-of-age at the relevant time. The worker also had a contused shoulder which was incompatible with the February incident. Having considered all the evidence as to the worker's continued work activities, including the salvaging and transporting of items even if assisted by mechanical means, as well as the delay in seeking medical attention and the discrepancies in the initial medical reporting, I conclude that there is insufficient evidence to establish that the February 18, 2013 slip-and-fall incident played a significant role in the development of the worker's rotator cuff tears.

[131] I note that the worker also sought to impugn the employer's motives by stating that a manager who was 'like a friend' had hired him to fire another employee. Once again, the worker's own further testimony undermined this account. The worker also claimed that a later employee had been hired to similarly get rid of him, and then stated that he didn't know that for sure. None of this testimony was corroborated by contemporaneous documents. I find it is prejudicial to a party who was not present to give countervailing evidence and has no relevance to the issue under appeal. Even if the circumstances alleged were used to assess credibility, the only reasonable conclusion would be that the worker agreed with and colluded with the employer in the purposes he describes.

[132] On a balance of probabilities, I would find that the worker does not have entitlement for a right shoulder injury in the claim under appeal.

DATED: May 29, 2020

SIGNED: D. McBey